

Pediatric Developmental Screening Report

M-CHAT Autism Spectrum Screening · Confidential Clinical Document

Patient Information

Patient Name	John Doe	Patient ID	JD-2026-001
Date of Report	April 29, 2026	Exam Type	Developmental Screening (M-CHAT)
Age / Gender	4 years · Male	Ethnicity	Asian
Country of Residence	United States	Informant	Parent
Jaundice at Birth	No	Prior Screening App	No

M-CHAT Screening Responses

Item	Question	Response
A1	Does the child look at you when you call their name?	Yes
A2	Does the child use pointing to show interest?	No
A3	Does the child engage in pretend play?	Yes
A4	Does the child show interest in other children?	Yes
A5	Does the child bring objects to show you?	No
A6	Does the child make eye contact with you?	Yes
A7	Does the child imitate your actions?	No
A8	Does the child respond to their name being called?	Yes
A9	Does the child use simple gestures?	No
A10	Does the child smile in response to your smile?	Yes

6 out of 10 items screened positive.

Clinical Observations

The child's parent reports delayed eye contact and limited use of pointing gestures. Repetitive hand movements are present, along with a markedly restricted range of toy interests. Language development appears delayed relative to age-matched peers; the child currently communicates using single words and basic gestures. No history of neonatal jaundice. No significant past medical history reported.

Behavioral Findings

Social interaction: Reduced social smiling and limited shared attention observed.

Communication: Diminished spontaneous use of words and gestures for communicative purposes.

Repetitive behavior: Hand flapping noted by parent during routine activities.

Clinical Impression

The screening profile reveals six positive items across social, communicative, and behavioral domains, which is consistent with characteristics associated with Autism Spectrum Disorder (ASD). The pattern of responses warrants further clinical investigation. A formal diagnostic evaluation by a pediatric neurodevelopmental specialist is strongly advised before any clinical conclusion is drawn.

Recommendations

1. Refer to a pediatric neurodevelopmental specialist for comprehensive diagnostic evaluation.
2. Initiate early intervention services pending evaluation outcomes.
3. Conduct a speech and language therapy assessment given observed communication delays.

Examining Clinician

Date: April 29, 2026